PUBLIC DIAGNOSTIC EXEMPLAR · 02 IN READING ORDER

NHS

/ PRIMARY CARE

STRUCTURAL DIAGNOSIS

Recurring Continuity Failure — from publicly observable evidence.

> *"The NHS has records. What it has not ensured is that they arrive where clinical decisions are made."*

ARTIFACT FAMILY

#	ARTIFACT	ACCESS
01	Translation Artifact	Public
02	**Diagnostic Artifact**	← *This document*
03	Redesign Executive Summary	Public
04	Full Redesign Artifact	Restricted

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CLASSIFICATION Public · 02 in reading order · derived from public evidence

EVIDENTIARY STANDARD	JUDGMENT BOUNDARY
This artifact distinguishes seven evidence tiers: **[1]** First-party NHS and government sources — stated promise, official workflow, contract and policy **[2]** Independent structural oversight — HSSIB, NAO, Public Accounts Committee **[3]** Consequence evidence — NHS Resolution data, coroners' Prevention of Future Deaths (PFD) reports **[4]** Workforce and collapse-node evidence — RCGP, BMA, Health Foundation **[5]** Patient and hidden-burden evidence — Healthwatch England **[6]** Academic and peer-reviewed evidence — BJGP, BMJ Open, Annals of Family Medicine, JMIR **[7]** Structural inference — the author's frameworks applied to the convergent evidence The governing diagnosis is a high-confidence structural inference [7] from convergent public evidence — not a claim about intent or individual failure. Full source documentation is in Section 14.	This artifact makes no claim about clinical negligence or individual clinician failure; institutional bad faith or intentional misconduct; unpublished NHS system logic; undisclosed contractual arrangements; or adjudicated legal liability. The condition described is architectural, not personal — consistent with decades of fragmented IT procurement and organisational record ownership, not with malice or incompetence. **Evidence discipline:** PFD reports record coroners' concerns; they are not findings of liability, and no single report is treated as proof of the diagnosis. NHS Resolution figures describe the NHS-wide claims environment; they are not treated as costs caused by the continuity failure.

SECTION 0

WHAT THIS DOCUMENT IS

This is a structural diagnosis from public evidence. It is not a legal brief, a policy manifesto, a technology procurement recommendation, a clinical guideline, or a redesign specification. It identifies the governing primitive, the collapse node, the function-collapse condition, the hidden-burden layer, and the dependency order for correction — derived entirely from publicly observable system behaviour.

The Full Redesign Artifact (Artifact 04 in this family) specifies one target-state architecture, its transition sequence, and its implementation logic. This document defines what must become true and in what order. It does not specify how.

RECOMMENDED READING ORDER	
Start here	Translation Artifact (Artifact 01) — plain-language explanation for all audiences
Full evidence	Diagnostic Artifact (Artifact 02) — this document
Replacement logic	Redesign Executive Summary (Artifact 03) — public board summary
Builder spec	Full Redesign Artifact (Artifact 04) — restricted; not published publicly

What this document does not say

- → It does not claim individual GP negligence, clinical failure, or bad faith by any clinician, practice, or NHS body.
- → It does not say the 10-minute consultation must be abolished. It says the consultation is the structural container in which the consequences of the primitive break are compressed.

- → It does not say GPs are failing. It says the architecture asks the GP consultation to carry four institutional functions at once without structural support for any of them.
- → It does not prescribe a technology vendor or EHR solution.
- → It does not claim access to unpublished NHS internal data, undisclosed contracts, or adjudicated legal findings.
- → It does not say all patients must see the same GP. It says operational memory must be present at the point of clinical consequence, whichever clinician the patient sees.

SECTION 0.1

PLAIN-LANGUAGE GLOSSARY

Technical terms used in this artifact, defined for non-specialist readers.

TERM	PLAIN MEANING
Operational memory	The usable clinical picture that must be in front of the clinician before the next decision — recent changes, outstanding results, unresolved symptoms, current medication. Not the whole record: the part the next decision needs.
Audit trace	The record of what happened, produced for compliance, defensibility and later review. Necessary, but a different object from operational memory.
Collapse node	The single point where too many system failures converge. Here: the short GP consultation.
Function fusion	Several structurally different jobs — sensing, interpreting, deciding and remembering — carried by one person at the same moment.
Hidden burden carriers	The people doing the system's memory work informally: patients repeating their history, carers keeping diaries, known GPs remembering, receptionists routing.
Event vs state	The NHS records events (appointments, referrals, discharges). A patient's condition is a continuous state that exists between those events.
Point of clinical consequence	The moment a decision with clinical consequences is made — prescribing, referring, discharging, reassuring.

SECTION 0.2

DIAGNOSTIC SPINE

The complete diagnostic finding in governed form. Every section of this artifact traces back to one or more rows of this table.

DIAGNOSTIC ELEMENT	FINDING
Governing primitive	Patient-carried operational continuity; the GP consultation as the fused sensing–interpretation–authority–memory node
Stated promise (paraphrased)	Joined-up, continuous care in which clinical records support decisions across the patient's whole NHS journey
Actual operating object	Organisation-held records; sharing request-based or after the fact; the patient's own account as the default continuity mechanism in the evidence reviewed

DIAGNOSTIC ELEMENT	FINDING
First structural break	Operational memory is not designed to be present at the point of clinical consequence. Continuity is reliable only where a human carries it.
Function collapse	Sensing, interpretation, authority and memory reconstruction converge in the short GP consultation without structural support for any of them
Missing function	Operational continuity retrieval at the point of clinical consequence — no contractual delivery mechanism or IT retrieval standard identified in public evidence
Hidden burden carriers	Patient (primary memory carrier); carer (secondary); known GP with accumulated knowledge (tertiary); receptionist / care navigator (quaternary)
Correction prerequisite	Name operational continuity as a structural requirement at national leadership level before any redesign
Proof condition	**Same pressure → different response.** Under the same workload and patient complexity, usable prior state is present before the decision, repeat unresolved presentations are surfaced, and patients no longer repeat their history as the default.

SECTION 1

EXECUTIVE SUMMARY

NHS primary care has a recurring continuity failure. Patients report repeating their history. Discharge information arrives late, incomplete, or not at all. The same unresolved symptoms can present across several consultations without being connected. Successive interventions — the National Programme for IT, the Summary Care Record, local shared care records, the NHS App, access reforms, ARRS workforce expansion and contractual continuity measures — have improved storage, access or capacity. The public evidence reviewed does not show any of them establishing that usable continuity arrives at the point of decision.

> GOVERNING DIAGNOSIS
>
> ***NHS primary care is record-rich and continuity-poor: it has built extensive audit trace without ensuring that operational memory is retrievable at the point of clinical consequence. The missing memory function has been carried by patients, carers, known GPs and informal practice staff — while the GP consultation carries sensing, interpretation, authority and memory reconstruction compressed into a short appointment.***

The condition can be stated simply: operational memory is not reliably available at the point of clinical consequence. The patient repeats their history because the system does not reliably retrieve it. Because memory is not structurally present, the GP must sense, interpret, decide and reconstruct prior state at the same time, in a short appointment, under full clinical consequence.

In 2023 the Health Services Safety Investigations Body (HSSIB) found no specific GP contract requirement to deliver continuity of care and no requirement for GP IT systems to surface information when patients return with unresolved symptoms [2]. The 2024/25 contract now requires continuity to be considered when practices decide how to respond to a patient, and the 2025/26 contract adds an incentive to identify patients who would benefit most [1]. These are real steps. Neither establishes a retrieval standard: a requirement that usable history is in front of the clinician when the decision is made.

Relational continuity was the human patch. For decades, the structural absence of operational memory was masked by patients seeing the same GP. The relational GP carried memory personally — social context, prior episodes, the symptom mentioned once. Public survey data show that kind of continuity declining [6]. The patch cannot be scaled by design; the memory function has to be built into the architecture.

The consequence environment is large. Coroners issued 713 Prevention of Future Deaths reports across all topics in 2024 [3], and The Guardian reported 36 coroners' warnings that year concerning NHS information sharing [3]. Delay or failure to diagnose featured in 43.5% of claims in the first year of the GP indemnity scheme [3]. Research associates lower GP continuity with more emergency admissions [6]. These figures show the scale of the environment in which the failure pattern appears; they are not offered as costs caused by it.

BOARD VERDICT

The NHS has built the audit layer and left much of the operational layer with the patient. The GP consultation is the collapse node. Until operational memory is structurally present before clinical consequence, new programmes remain liable to relocate the same failure under a new name.

PROOF CONDITION

Same pressure → different response. The redesign is real only when, under the same workload and patient complexity, usable prior state is present at the point of clinical consequence, repeat unresolved presentations are surfaced before the decision, and patients no longer repeat their history as the default.

SECTION 2

ENGAGEMENT SCOPE

System under analysis

NHS primary care continuity at the GP-led front door, and across the GP → referral → secondary care → discharge → GP return pathway.

Included in scope

GP consultation (first contact and repeat presentation); referral generation and receipt; specialist handover; discharge back to the GP; record retrieval at the point of consultation; patient and carer continuity burden; NHS information architecture (Summary Care Record, local shared care records, GP Connect, NHS App, and the proposed Single Patient Record).

Excluded unless directly relevant

Broad NHS finance reform; hospital management generally; unrelated digital health policy; international health systems (referenced only as structural comparators after the governing diagnosis is formed).

Framing note

The mismatch diagnosed here is not a moral failure. It is consistent with decades of fragmented IT procurement, organisational record ownership and reform programmes measured by what they built rather than by what arrived at the decision. Clinicians and practice staff are, in this reading, the people compensating for the architecture — not the cause of the failure.

Evidence basis

This diagnosis is based entirely on publicly available materials, reviewed through October 2026 [1]–[7]. Primary sources include: NHS England and DHSC materials, including the 2024/25 and 2025/26 GP contract changes, the 10 Year Health Plan (July 2025) and the NHS Modernisation Bill announced in the May 2026 King's Speech; HSSIB investigation reports (2023; 2025); NHS Resolution annual and CNSGP data; Public Accounts Committee reporting on the National Programme for IT; RCGP surveys; Health Foundation workforce projections; the Healthwatch England national survey (n=1,800, March 2025); coroners' PFD reports and the Ministry of Justice Coroners Statistics 2024; and peer-reviewed literature (BJGP, BMJ Open, Annals of Family Medicine, JMIR Medical Informatics). Section 14 maps each claim to its sources.

Known fixed constraints

The GP contractor model; demand-driven appointment availability; the short standard consultation; the multi-vendor EHR landscape; the governance, privacy and commercial environment surrounding NHS health data; and a changing institutional landscape — the proposed NHS Modernisation Bill would allow NHS England to be abolished. Roles named in this artifact refer to current bodies or their successors.

SECTION 3

FAILURE PATTERN RECONSTRUCTED FROM PUBLIC EVIDENCE

The following represents the operating problem as the public record reconstructs it, before structural translation. These statements are derived from convergent public sources [1][2][4][5], not from undisclosed internal communications.

CLAIM TYPE	HOW THIS ARTIFACT TREATS IT
First-party NHS and government statements [1]	Direct evidence of the stated promise, contract and policy. The most reliable source for what the system says it does.
Independent oversight [2]	Investigation and audit findings. Treated as strong structural evidence for the period each report covers.
Consequence evidence [3]	Claims data and coroners' reports. Treated as the consequence environment — consistent with the diagnosis, not proof of it, and not findings of liability.
Workforce and patient evidence [4][5]	Survey evidence of the burden as experienced. Treated as convergent pattern evidence, not as individual fact.
Structural diagnosis [7]	Inference from convergence across all tiers. Clearly marked. Not a claim about internal intent or undisclosed systems.

- → **Access and workload:** practices report unsustainable workload; much time goes on administration; many patients cannot see a familiar GP [1][4][5].
- → **Record fragmentation:** GPs do not reliably have the information they need, especially after hospital discharge. Discharge correspondence can be delayed, incomplete, or not sent. Different organisations use different systems [2].
- → **Digital progress and persistent failure:** substantial investment has gone into the NHS App, shared care records, GP Connect and the Summary Care Record. Documentation and access have improved. The continuity pattern persists in the evidence reviewed [1][2].
- → **Unresolved presentations:** patients with long-term conditions can have several appointments without their care being coordinated; the same issues return; diagnoses can be delayed [2][3].

→ **Contractual measures without a retrieval mechanism:** the 2024/25 and 2025/26 contracts introduce continuity considerations and incentives. They are difficult to deliver when the workforce is stretched, practices are large, and the digital infrastructure does not surface continuity at the point of care [1][2].

These are not treated as isolated complaints or population statistics. Their recurrence across record access, clinical handover, workforce sustainability and patient experience is the structural signal examined in this diagnostic.

SECTION 4

EARLY STRUCTURAL SIGNALS

Four structural signals are visible from the public record before the formal diagnosis is applied.

1. The system promises continuity but has had no structural mechanism to guarantee it.

HSSIB's 2023 investigation found no specific GP contract requirement for continuity of care and no requirement for GP IT systems to consider continuity or surface information when patients return with the same unresolved issue [2]. The government's response points to the 2024/25 contract, which requires continuity to be considered when responding to patients; the 2025/26 contract adds an incentive to identify patients who would benefit most [1]. Continuity has moved from absent to encouraged. The public evidence does not yet show it as a structural requirement with a defined delivery mechanism at the point of decision.

2. Corrections have mainly addressed documentation and access, not retrieval at the decision.

The National Programme for IT did not deliver its central aim of a national electronic patient record, although components such as the Summary Care Record survived [2]. The Summary Care Record provides a minimal cross-organisational view. Local shared care records provide professional read access that varies by region [1]. The NHS App gives patients access to their GP record [1]. Each improved the institution's ability to store, share or display information. The public evidence reviewed does not show any of them measured by whether usable history was in front of the clinician at the moment of decision [2][7]. The proposed Single Patient Record is the next programme in this line; whether it changes the outcome depends on what it is measured by.

3. Patients and carers are filling the gap.

In a 2025 national survey, 23% of adults had noticed inaccuracies or missing details in their records; of those, 26% said they must repeat their patient history, and 20% had struggled to convince staff of something in their history because it did not appear in their records [5]. HSSIB's 2025 investigation into discharge communication heard of an "over reliance" on patients and families when discharge information had not arrived [2].

> SHARPEST STRUCTURAL SIGNAL
>
> ***When the system's own information does not arrive, the patient's memory becomes the fallback. That is not patient empowerment. It is the memory function being carried, informally and without design, by the person least equipped to carry it under clinical pressure.***

4. The consequence layer is patterned, not random.

The Guardian reported 36 coroners' warnings in 2024 concerning the sharing of NHS patient information [3]. Research associates lower GP continuity with higher emergency admission rates [6]. Coroners continue to raise record-sharing with national bodies; in June 2026 the government responded to one such report by pointing to the Single Patient Record programme [1][3]. These consequences cluster around the same structural condition. They are consistent with the diagnosis; they are not offered as proof of it.

SECTION 4.1

COMMON MISDIAGNOSES

The following explanations are frequently proposed for the continuity failure. Each addresses something real, but each sits downstream of the break and does not by itself reach it.

MISDIAGNOSIS	WHY IT IS INSUFFICIENT
GP workforce shortage	Capacity addresses throughput, not memory retrieval. More GPs working with the same record architecture still lack reliable prior history at the point of care.
IT interoperability problem	Interoperability is necessary but not sufficient. Systems that can exchange data (SCR, GP Connect) coexist with continuity failure at the consultation. Interoperability transports data; retrieval determines whether the right clinical memory appears before the decision.
Discharge summary delay	Treating this as a communication delay ignores that the handover has no guaranteed retrieval step. Faster summaries without guaranteed surfacing at the next consultation do not correct the break.
Patient empowerment through record access	Read access for patients does not put operational memory in the clinician's workflow. Presented as the fix, it shifts the retrieval burden to the patient rather than correcting the absence.
ARRS workforce expansion	Adding role types to a pooled appointment list increases capacity but can further fragment relational continuity. Capacity and continuity are different requirements.
Contractual continuity measures	A contract consideration or incentive without a retrieval standard does not guarantee that usable history appears when a returning patient presents to any clinician.

SECTION 5

SYSTEM BOUNDARY MAP

The system operates as a distributed record field with no central continuity node. Records exist across organisational silos. Access is conditional, delayed or request-based rather than native. Continuity is reconstructed at the consultation by whoever is present.

Primary actors

ACTOR	ROLE	POSITION IN SYSTEM
Patient	Carries own continuity history; repeats it at encounters; fills the memory gap the architecture leaves	De facto primary memory carrier — undesigned, uncompensated

ACTOR	ROLE	POSITION IN SYSTEM
Carer / family member	Maintains medication logs, appointment diaries and care histories across transitions	Secondary memory carrier — load-bearing, structurally invisible
GP (known / relational)	Clinical judgment with accumulated patient knowledge; carries social context and prior episodes personally	Collapse node with relational continuity available
GP (pooled / unknown)	Clinical judgment without prior knowledge of the patient; reconstructs history from fragments and the patient's account	Collapse node — full memory reconstruction required
Receptionist / care navigator	Informal triage, complexity flagging, routing returning patients toward known clinicians where possible	Informal sensing and continuity brokering — undesigned
NHS 111	Remote triage and urgent referral; limited record access	Parallel sensing node with limited continuity access [3]
Secondary care / specialist	Assessment and treatment in a separate EHR	Separate record silo
Discharge summary	Main route for clinical information from hospital to primary care	Handover mechanism — can be delayed, incomplete or not sent [2]
Summary Care Record (SCR)	Medications, allergies, adverse reactions — national minimal record	Audit-grade memory layer [1]
Local shared care records	Cross-organisational professional read access; coverage varies by region	Partial operational layer; national support programme (ConCR) scheduled to finish March 2026 [1]
NHS App	Patient-facing access to the GP record	Partial patient visibility layer [1]
Single Patient Record (proposed)	Planned combined health and care record, viewable by patients in the NHS App	Proposed national layer; retrieval at the decision not yet specified in public evidence [1]

DECISION POINTS WHERE STRUCTURAL FAILURE CONCENTRATES

1. Whether usable clinical history is present when the GP opens a returning patient's record [2][5]
2. Whether a repeat unresolved presentation is surfaced before the decision — HSSIB found no requirement for GP IT systems to do this (2023) [2]
3. Whether discharge information has arrived and been processed before the follow-up appointment [2]
4. Whether the consulting clinician has any prior knowledge of the patient [4][5]

SECTION 6

WORKFLOW AND PIPELINE TRACE

The documented operating sequence:

1. Patient develops a symptom; contacts the practice by phone or online system [1][4]
2. Receptionist or care navigator routes to an available appointment — not necessarily with the patient's usual GP [1][4]
3. GP opens the consultation; the record shows GP notes, the SCR summary and any processed correspondence [1][2]
4. GP reconstructs clinical history from the partial record and the patient's account — often the patient is the main continuity source [2][5]
5. GP senses presenting symptoms, interprets them against the reconstructed history and makes a consequential decision [2][4]
6. GP documents the encounter — for clinical handover and for audit and defensibility [4][6]
7. If a referral is made: the secondary care episode takes place in a separate EHR [2]
8. Secondary care produces a discharge summary — expected within 24 hours under the NHS Standard Contract; in practice it can be delayed, incomplete or not sent [1][2]
9. Summary received at the practice; reviewed and processed by staff — it may not appear in the consultation view until then [2]
10. Patient returns for follow-up; the cycle repeats from step 3 with no structural guarantee that discharge information is present [2][5]

Critical structural observations

→ **No step guarantees operational memory before the decision.** No step in the public workflow ensures operational memory is present before the clinical decision is made [2][7]. The workflow assumes the clinician will reconstruct what the system does not retrieve.

→ **Discharge information can arrive after the patient.** HSSIB's 2025 investigation found discharge summaries created but not sent; in one serious incident the problem had been running for around a year. GPs described relying on patients and families when information had not arrived [2].

→ **The pooled list makes relational continuity harder by default.** A patient presenting several times in a few months — often the pattern most likely to indicate an unresolved diagnostic need — is less likely to see the same GP each time. Survey data show continuity falling over the past decade [6].

→ **Repeat unresolved presentations are not reliably surfaced.** The same symptom, coded differently across consultations with different clinicians, need not trigger a flag. HSSIB recommended updating GP IT standards to surface such patterns; NHS England's response noted existing alerting capabilities and committed to review them [2].

SECTION 7

FUNCTIONAL DIAGNOSIS

Applying the Four-Function Law [7]: institutions fail under scale when sensing, interpretation, authority and memory remain fused or misaligned at the point of consequence.

FUNCTION	FORMAL LOCATION	ACTUAL LOCATION	CONDITION	STRUCTURAL CONSEQUENCE
Sensing	NHS 111, digital triage, practice reception	GP in consultation — absorbing signals from the patient's account, record fragments and presenting symptoms	Misaligned — sensing happens at the collapse node, after the pre-consultation opportunity has passed	Signal quality depends on patient recall and record availability. Weak signals — an unresolved pattern across encounters with different GPs — are hard to see [2][5]
Interpretation	Clinical frameworks, NICE guidance, specialist input	GP in a short consultation, reconstructing history while forming a judgment	Fused with authority — interpretation and decision happen at the same moment	Judgment is formed from incomplete information. HSSIB found GPs in practices with a continuity system had more time to process information [2]
Authority	GP as registered clinician	GP in a short consultation — decides, prescribes, refers or discharges	Fused with sensing and interpretation — no structural separation between signal reception and consequential action	Delay or failure to diagnose featured in 43.5% of claims in the GP indemnity scheme's first year (2019/20, 401 cases), with a caution against firm conclusions [3]. Consistent with an authority node receiving degraded inputs; not evidence that continuity failure caused those claims [7]
Memory	GP record, SCR, shared care records	Patient (verbal history); carer (medication logs); GP (accumulated knowledge, known patients only)	Not reliably present at the point of need — carried by the patient without design, recognition or compensation	The memory function falls to the person least equipped to carry it. When that person cannot, public evidence identifies no reliable structural fallback [2][5]

CRITICAL FUSION AND ABSENCE [2][7]

The critical fusion: sensing, interpretation and authority converge in the GP consultation, and the short appointment leaves little structural space to separate them. The critical absence: operational memory is not reliably present at the point of clinical consequence. The critical inversion: investment in documentation, audit and access has grown without a matching measure of whether usable continuity arrives at the decision.

AUDIT TRACE VS OPERATIONAL MEMORY — NAMED DISTINCTION

Audit trace records what happened — produced for compliance and defensibility. Operational memory surfaces what must be known before the next decision — produced for clinical use. The NHS has built the first extensively. The public evidence does not show it building the second. They are structurally different objects requiring different architecture, custody and incentives.

SECTION 8

STRUCTURAL FAILURE POINT

DIAGNOSTIC LAW

A health system fails at the consultation when the information required for clinical judgment is not present at the point of clinical consequence. The public evidence reviewed is consistent with NHS primary care exhibiting this failure pattern.

The first structural break

The system is not designed to deliver operational memory to the point where clinical decisions are made. Continuity is reliable only where a human carries it.

This is not the delayed discharge summary — that is a downstream consequence. It is not the short appointment — that is the container in which the consequences of the break are compressed. It is not GP burnout or workforce shortage — each is a further downstream effect. The break is consistent with decades of fragmented IT procurement, organisational record ownership, and digital reform that prioritised documentation over retrieval [2][7].

Data without context. The GP record is a stack of facts, not a narrative of clinical state. The system can expose fragments without constructing the clinical state the decision needs. Operational-grade retrieval is not access to a PDF of a discharge summary. It is the relevant clinical change since the last visit — outstanding investigations, unresolved presentations, medication changed in A&E — surfaced in usable form before clinical consequence begins.

Data gravity. Clinical data tends to stay in the silo where it was created. The public evidence identifies no national mechanism that pulls the relevant data toward the point of decision. The information often exists. The force that would bring it to the decision is missing.

The three capability modes

MODE	DESCRIPTION	HOW THE NHS HANDLES IT
Execution reliability	Consistent appointment delivery, prescription processing, routine referral	Actively managed and measured. This is what access and throughput metrics capture. [1][4]
Diagnostic capability	Pattern recognition across encounters; escalation of unresolved presentations; detection of weak signals over time	Present for patients with relational continuity to a known GP. Weak across the pooled list and care transitions. [2][4]
Continuity retrieval	Surfacing usable clinical history at the point of consequential decision	No contractual delivery mechanism or GP IT retrieval standard identified in public evidence; continuity now a contract consideration and incentive. [1][2]

SHARPEST STRUCTURAL CLAIM — PUBLICLY SUPPORTED [2][7]

The third capability — the one that would connect five appointments, surface the medication changed in A&E, and flag the symptom coded differently three times — is weak not mainly because the data does not exist. In the evidence reviewed, it is weak because no structural mechanism has been identified that surfaces that data at the moment it is needed.

Evidence of the break

- **HSSIB (2023) [2]:** no specific GP contract requirement for continuity; no requirement for GP IT systems to surface repeat unresolved presentations.
- **HSSIB (2025) [2]:** discharge summaries created but not sent; in one serious incident the problem had run for around a year; over-reliance on patients and families when information had not arrived.
- **Healthwatch England (2025) [5]:** of the 23% of adults who had noticed errors in their records, 26% said they must repeat their patient history and 20% had struggled to convince staff of something in their history.
- **JMIR Medical Informatics (2023) [6]:** in 2017/18, patients attended a trust using a different EHR system from their previous attendance on over 11 million occasions; the author infers that for 9.1% of acute presentations, clinicians were left uninformed about recent history.

Publicly documented cases involving record access or handover

Coroners' PFD reports record concerns about future risk. They are not findings of liability, and each death below had several contributing factors.

- **Isaac Onyeka, aged 3 (East London, 2024) [3]:** the coroner found a different NHS 111 disposition would have been reached had the health adviser known of his Down's syndrome diagnosis; the report also records risk factors not considered at a GP assessment.
- **Regan Smith, aged 11 (Suffolk, 2024) [3]:** handover between ambulance and emergency department relied on verbal handover; ambulance and hospital IT systems were not directly compatible; a key blood glucose reading was not passed on.

SECTION 9

COMPENSATION AND HIDDEN BURDEN

NHS primary care appears to function. Appointments are booked, prescriptions issued, referrals made. The strongest structural reading is that it functions because a set of informal human carriers performs continuity functions the architecture does not formally support [2][5][7].

Primary carrier — the patient

The patient is the de facto memory function of primary care [2][5]. Patients report repeating their history because the system does not reliably retrieve it: of adults who had noticed record errors, 26% said they must repeat their patient history [5]. When the patient cannot carry this burden — because they are unconscious, acutely unwell, cognitively impaired, or unaware of their own clinical history — public evidence identifies no reliable structural fallback. The cases in Section 8 show what that can look like [3].

Secondary carrier — the carer

Where the patient cannot carry the burden alone, the carer does: medication logs across several prescribers, appointment diaries, context across transitions that the formal record does not reliably bridge

[2][5]. HSSIB's 2025 investigation heard of over-reliance on patients and families when discharge information had not arrived [2]. This labour is unpaid, unrecognised, and structurally load-bearing.

Tertiary carrier — the GP with accumulated knowledge

For patients with a consistent relationship with a named GP, that GP carries the memory function personally — social context not in the record, family history mentioned once, a previous episode coded under a different symptom [4][6]. This knowledge is clinically valuable, hard to replace, and structurally precarious. It exists only for patients with relational continuity, disappears when the GP retires, moves or is unavailable, and does not travel across care transitions [4].

Quaternary carrier — the receptionist and care navigator

The receptionist performs informal triage: identifying complex patients, routing returning patients toward known clinicians where possible, flagging concerns to clinical staff [4]. This is undesigned, uncompensated as clinical work, and structurally load-bearing. It operates largely outside the clinical record.

STRUCTURAL CONSEQUENCE

When any carrier withdraws — the carer can no longer keep the log, the patient cannot recall their history, the known GP is unavailable — the structural failure can surface as a clinical event. The system appears functional because these carriers absorb the gap. Their labour is not visible in the budget, productivity and reform measures reviewed.

SECTION 10

GOVERNING DIAGNOSIS

The recurring failure is not best explained as a GP capacity problem, a digital investment shortfall, or a workforce crisis on its own. The structural reading — derived from convergent public evidence across the record architecture, the clinical workflow, the consequence layer and the patient experience pattern [1]–[7] — is this:

THE GOVERNING DIAGNOSIS — COMPLETE STATEMENT

The NHS has not failed to create records; it has not ensured that those records arrive as usable clinical continuity at the moment a consequential decision is made. The reforms reviewed here — the National Programme for IT, the Summary Care Record, shared care records, the NHS App and contractual continuity measures — improved documentation, access or sharing without establishing that usable continuity reaches the decision. The result is a system that is record-rich and continuity-poor: it holds more data about patients than at any previous point, while the evidence shows that data often failing to arrive in usable form where it is needed. While the memory function remains externalised onto patients, carers and known GPs, downstream fixes remain liable to relocate the same failure rather than remove it. The proposed Single Patient Record is the next test: whether it is measured by data assembled, or by usable continuity at the point of decision.

EVENT-BASED VS STATE-BASED — THE ARCHITECTURAL MISMATCH

The NHS architecture is optimised for events — the appointment, the referral, the discharge. It is weak on state — the patient's longitudinal clinical condition across encounters and transitions. Managing a state-bearing person through an event-based ledger produces continuity failure unless something carries the state. Today, that something is usually the patient.

SECTION 10.1

PRIMITIVE MISMATCH TABLE

DIMENSION	STATED	ACTUAL
Primitive	Joined-up care in which records support clinical decisions across the patient's whole NHS journey	Each organisation holds its own records; sharing is request-based or after the fact; the patient's account is the default continuity mechanism
What is promised	Continuity of care at the point of need	Audit-grade trace of what happened; operational continuity reconstructed at each encounter, often by the patient
What is purchased	Clinical judgment supported by retrievable patient history	Clinical judgment formed from what the patient can remember, overlaid on a fragmented record
Who succeeds	All patients	Patients with relational continuity to a known GP, a carer who keeps logs, or the ability to reconstruct and assert their own history
Who bears the structural cost	—	The informal carrier layer: patient, carer, known GP, care navigator — unrecognised, uncompensated, load-bearing
What is measured	Appointment volumes, access times, digital adoption, documentation compliance	Output metrics that do not test the operational continuity layer
What is not measured	—	Whether usable clinical continuity was present at the point of the consequential decision — the metric that would reveal the failure
Commercial structure	Open, interoperable data flow supporting joined-up care	Retrieval across organisations depends on supplier systems, interfaces and agreements. No incentive was identified in public evidence that rewards cross-organisational retrieval at the point of decision; the government has said suppliers will need to meet common data-sharing standards [1] [7]

SECTION 10.2

WHY THE MISMATCH IS STRUCTURAL, NOT INCIDENTAL

The mismatch is structurally consistent with a collision between forces that have not been reconciled: fragmented IT procurement that produced an estate of different EHR systems; organisational record ownership, in which each body holds its own records and sharing is an after-the-fact join; and a

commercial environment in which cross-organisational retrieval depends on supplier interfaces and agreements [2][7]. Together, these make operational continuity hard to achieve within the current architecture without a change at the primitive level.

Incentive misalignment [7]. The current environment rewards documentation density — useful for billing, defence and audit — and has not been counterbalanced by a requirement to deliver operational-grade retrieval at the point of decision. This is a statement about incentive structure, not about the intent of any supplier or NHS body.

The public record does not show this condition resolving on its own. Until the primitive is named and operational memory is placed at the point of clinical consequence, the failure remains liable to be regenerated by the same structural conditions that sustain it.

SECTION 10.3

VALIDATION TEST

If the NHS implemented the diagnostic refusal conditions in Section 12 and required that returning-patient consultations surface usable continuity — including outstanding discharge information and prior unresolved presentations — would the remaining friction be explained by execution quality rather than structural memory absence? If yes, the diagnosis holds.

Measurable signals that would test the diagnosis without full redesign:

- → **Expose discharge summary status to GPs in real time.** If administrative chasing falls but patient-reported history repetition does not, the binding constraint is retrieval at the decision, not transmission speed — diagnosis supported.
- → **Pilot surfacing of repeat unresolved presentations in one ICB area.** If diagnostic delay for multi-presentation patients falls, the missing surfacing layer was a binding constraint — diagnosis supported.
- → **Measure presence at the moment of decision.** Record the proportion of returning-patient consultations where relevant discharge information is present and visible when the clinician opens the record. Set the threshold from the baseline; a persistently low rate across practices points to a structural rather than individual cause.

Falsification condition: if patients still routinely repeat their history after usable operational memory is reliably present at the point of care, revisit whether the fixed constraints in Section 2 prevent full correction or whether the mechanism has been misidentified.

STRONGEST FALSIFIER

If usable continuity is reliably present before the decision — discharge information, outstanding results, repeat presentations surfaced — yet the same history-repetition, diagnostic-delay and carer-burden pattern persists under comparable demand, the diagnosis must be revised. The diagnosis holds if and only if restoring operational memory at the point of consequence changes the symptom pattern.

SECTION 11

COST OF CONTINUATION

A directional assessment from public evidence, not internal NHS financial data. The figures below show the scale of the consequence environment in which the continuity failure pattern appears. They do not show that the continuity failure causes these costs.

Class 1 — Consequence environment

INDICATOR	SCALE	SOURCE
Clinical negligence payments (all NHS Resolution schemes)	£2.8 billion paid in 2023/24 across all schemes — an NHS-wide figure, dominated by hospital claims	NHS Resolution, Annual Report 2023/24 [3]
GP indemnity claims (CNSGP)	2,382 claims in 2023/24, up 9% on 2022/23	NHS Resolution, Annual Report 2023/24 [3]
Delay / failure to diagnose (GP)	Featured in 43.5% of CNSGP cases in the scheme's first year (2019/20, 401 cases); NHS Resolution cautions that no firm conclusions can be drawn from one year	NHS Resolution, CNSGP year-one report (August 2022) [3]
Prevention of Future Deaths reports	713 issued in 2024 across all topics, up 25% on 2023	Ministry of Justice, Coroners Statistics 2024 [3]
Coroners' warnings on NHS information sharing	36 in 2024, as reported in November 2024	The Guardian, November 2024 [3]
National Programme for IT	Forecast total cost £9.8 billion; central aim of a national electronic patient record not delivered	DH statement 2013; Public Accounts Committee, September 2013 [2]

Class 2 — Structurally associated burden

INDICATOR	SCALE	SOURCE
Emergency admissions and continuity	Older patients with the least continuity had more than twice the odds of emergency admission of those with the most (odds ratio 2.32); a 2025 systematic review associates higher GP continuity with fewer admissions and emergency department visits	Annals of Family Medicine (2017); BJGP (2025) [6]
Presentations without recent history	Over 11 million occasions in 2017/18 where patients attended a trust using a different EHR system from their last; the author infers 9.1% of acute presentations	JMIR Medical Informatics (2023) [6]
GP workforce strain	76% of GPs say excessive workload compromises patient safety; 42% say they are unlikely to be working in general practice in five years	RCGP GP Voice survey (July 2024); RCGP poll (September 2024) [4]
GP workforce projection	Shortfall of around 8,800 FTE GPs by 2030/31 under current policy (about 1 in 4 posts); nearly 1 in 2 in the pessimistic scenario	Health Foundation (2022) [4]

Class 3 — Invisible patient and carer labour

INDICATOR	SCALE	SOURCE
Patients repeating their history	26% of those who had noticed record errors (23% of 1,800 adults surveyed)	Healthwatch England (2025) [5]
Patients unable to evidence their history	20% of those who had noticed record errors struggled to convince staff of something in their history	Healthwatch England (2025) [5]
Families as fallback	Over-reliance on patients and families when discharge information has not arrived	HSSIB (2025) [2]

The compounding dynamic — recurrence while the primitive remains [7]

A structural hypothesis to test, not an established causal sequence.

STAGE	MECHANISM	STRUCTURAL ORIGIN
1	Workforce pressure increases function fusion per consultation	Each GP carries more sensing, interpretation, authority and memory reconstruction per session
2	More fusion → more clinical risk	Authority node operating on degraded inputs under greater time pressure
3	More clinical risk → more claims and coroner concern	Delay in diagnosis can intensify where prior state is missing
4	More claims → more defensive documentation	Audit trace grows; clinical time shrinks further
5	More burden → more workforce exit	The collapse node becomes harder to sustain
6	More exit → return to Stage 1	Recurrence remains structurally likely while the memory function is unchanged

Structural upside if corrected: an architecture that places usable operational memory at the point of clinical consequence is designed to reduce diagnostic delay for multi-presentation patients, emergency admissions linked to discontinuity, GP reconstruction time, and the burden carried by patients and carers. Those effects require measurement under the proof conditions; they are not guaranteed by implementation alone.

SECTION 12

CORRECTION DEPENDENCY ORDER

This is a dependency order, not a list of recommendations. Right fixes in the wrong sequence still fail. This section identifies what must stop and what must become true before valid correction can hold. The Full Redesign Artifact (Artifact 04, Section 10) expands this diagnostic order into one implementation sequence.

Diagnostic refusal conditions

These are refusal conditions, not negotiable starting points for redesign conversations. Behaviours that reproduce the failure stop immediately. Structures that reproduce it stop before their replacement becomes the official path. Nothing scales while a refusal condition is still active. All four below are behaviours, so all four stop immediately.

STOP	WHY	FAILURE IF NOT STOPPED
Measuring digital programmes by adoption rather than by whether usable continuity reaches the decision [1][2]	Adoption metrics reward the audit layer. The operational layer stays unmeasured.	Programmes appear successful while the continuity failure persists
Treating discharge-summary failure as a communication problem [2]	HSSIB's 2025 findings describe system design and configuration failures, not only messaging delays.	Summaries get faster without guaranteed surfacing at the next consultation
Treating extra appointment capacity (including ARRS) as a continuity improvement [1][4]	Capacity and continuity are different requirements. More clinicians in a pooled list can further fragment relational continuity.	More appointments; function fusion per consultation unchanged
Presenting patient record access as a substitute for the clinician having the information [1][5]	Patient access is valuable, but as the fix it redistributes the retrieval burden to the patient instead of correcting the absence.	The externalisation is endorsed rather than corrected; a new digital layer formalises the old failure

These are the four diagnostic-level refusal conditions. The six-item implementation Vanish List is in Artifact 04, Section 8.

Correction sequence — five diagnostic conditions

These are the conditions that must become true, in dependency order. They are diagnostic conditions, not implementation moves, and they do not depend on any particular technology. Owners show where each condition sits in the current system; named bodies are exemplar allocations — the architectural requirement is an authority above local pressure.

Condition 1 — Continuity named as a requirement (prerequisite for everything else)

Name operational continuity as a structural requirement.

Leadership must define what operational continuity means: who holds the patient's continuity at the point of care, under what conditions it is retrievable, who may add to it, and the minimum standard for its presence when a consequential decision is made. Without this declaration, later reforms are interpreted through the existing frame [7].

Owner (exemplar)	National leadership — NHS England (or successor body) and DHSC
Risk if skipped	Downstream changes are read through the audit frame and produce the same partial fix
Redesign reference	Artifact 04, Section 5 (corrected primitive) and Section 10, Move 1

Condition 2 — Operational record separated from the audit record (follows from Condition 1)

Separate operational continuity from audit trace.

The audit record exists for compliance and defensibility. Operational continuity must be built for clinical use — retrievable at the next point of care, so the next clinician can act without reconstruction. These need different structures, custody arrangements and incentives.

Owner (exemplar)	National digital leadership and record standards bodies
Risk if skipped	Continuity stays fragmented even as individual systems become better documented
Redesign reference	Artifact 04, Section 4 (truth kernel) and Section 10, Moves 1 and 1b

Condition 3 — Usable history in front of any clinician before the decision (follows from Condition 2)

Make the relevant prior state present before the consultation begins.

Whichever clinician the patient sees, the relevant prior state — recent changes, outstanding investigations, current medication, and repeat unresolved presentations — must be present and readable before the decision. HSSIB recommended that GP IT surface repeat unresolved presentations; this condition generalises that recommendation [2].

Owner (exemplar)	GP IT standards owner; practices; clinical safety leads
Risk if skipped	Diagnostic delay persists for multi-presentation patients despite better records
Redesign reference	Artifact 04, Section 10, Moves 2 and 3

Condition 4 — Outcomes and handovers carried forward (follows from Condition 3)

Close the loop after every consequential encounter and transition.

Consultation outcomes, discharge information and referral status must reach the next decision in usable form — not left as delayed correspondence. The handover mechanism must guarantee arrival before the follow-up, not only transmission.

Owner (exemplar)	Integrated care boards and secondary care trusts; practices
Risk if skipped	Discharge information improves in volume without guaranteed presence at the next consultation
Redesign reference	Artifact 04, Section 10, Move 4; Move 7 for institutional data such as discharge summaries

Condition 5 — It holds when the patient cannot carry it (follows from Condition 4)

Make continuity survive the hardest cases.

Continuity must be available when the patient is unconscious, acutely unwell, cognitively impaired or digitally excluded — through emergency access, delegated carer access and non-digital fallback. A correction that works only for confident, well patients has not removed the dependency on the patient.

Owner (exemplar)	National leadership; ICBs; practices
Risk if skipped	The cases in Section 8 remain structurally possible
Redesign reference	Artifact 04, Section 10, Moves 5 and 6

DEPENDENCY SUMMARY

Condition 5 depends on Condition 4. Condition 4 depends on the presence established by Condition 3; Condition 3 depends on the separated operational record in Condition 2; Condition 2 depends on the requirement declared in Condition 1. Programmes that attempt access or presentation changes without Conditions 1 and 2 in place are the pattern this diagnosis describes.

Artifact 04 specifies one route to these conditions: a patient-held operational continuity layer built in seven moves (with Move 1b for privacy and security). It is designed to begin meeting Conditions 1–5 without waiting for institutional change, and it leaves institutional routes open.

DIAGNOSTIC CONDITION	ARTIFACT 04 MOVE(S)
1 — Continuity named as a requirement	Section 5 corrected primitive; Move 1 (truth kernel defines the continuity object and its minimum standard)
2 — Operational record separated from audit	Move 1 (append-only continuity record built for retrieval); Move 1b (privacy and security)
3 — Usable history before the decision	Move 2 (symptom event layer); Move 3 (consultation packet, including repeat presentations)
4 — Outcomes and handovers carried forward	Move 4 (outcome append); Move 7 (institutional data such as discharge summaries, once recognised)
5 — Holds when the patient cannot carry it	Move 5 (emergency access); Move 6 (carer access); non-digital fallback throughout

Acceptance criteria: what must be true before implementation-level redesign

In a live engagement, redesign begins only after the governing diagnostic frame has been formally resolved and locked. The following conditions describe the decision boundary for this exemplar; they are not represented as a universal signature protocol.

1. Leadership resolves the primitive: operational continuity is a structural requirement, not a desirable outcome.
2. The diagnostic refusal conditions are accepted as constraints on the redesign.
3. The five-condition diagnostic dependency order is accepted.
4. Baselines are agreed before change: patient-reported history repetition, GP time on reconstruction, discharge information present before follow-up, emergency admissions for repeat presenters.
5. Where a Full Structural Engagement is commissioned, Artifact 04 provides the implementation-level replacement architecture.

SECTION 13

ROLE-LEVEL DIAGNOSTIC IMPLICATIONS

Structural translation of the diagnosis into implications by role. These follow the dependency sequence in Section 12. They are diagnostic implications, not implementation instructions; the system object model, transition logic and builder handoff are in the Full Redesign Artifact (Artifact 04).

ROLE	DIAGNOSTIC IMPLICATION	MUST STOP ASSUMING	IMMEDIATE DIAGNOSTIC CHECK	REDESIGN REFERENCE
National leadership (NHS England or successor; DHSC)	The primitive must be named before redesign can hold. New digital programmes should define how retrieval at the point of decision will be measured — not adoption. Say plainly that the current architecture leaves memory work with patients.	That more data assembled means more continuity delivered.	Is any programme measured by whether usable history was present at the decision?	Artifact 04 § 5; § 10 Move 1

ROLE	DIAGNOSTIC IMPLICATION	MUST STOP ASSUMING	IMMEDIATE DIAGNOSTIC CHECK	REDESIGN REFERENCE
Digital and technology leadership	Expose discharge information status to GPs in real time. Separate operational continuity from audit trace. Do not present patient-facing access as a substitute for clinician-side retrieval.	That interoperability alone produces continuity.	Can a GP see, on opening the record, whether discharge information has arrived?	Artifact 04 § 10 Moves 1–3; § 16
GP practices and clinical safety leads	Identify patients carrying the highest continuity burden: repeat presenters, post-discharge patients, multiple long-term conditions. The informal work of receptionists and care navigators is load-bearing, not supplementary.	That history reconstruction is normal clinical work.	How many consultations each week begin with verbal reconstruction?	Artifact 04 § 7; § 10 Moves 3–4
Integrated care boards	Use population data to find where discontinuity is producing repeat emergency admissions. Close the loop between discharge and GP follow-up.	That discharge is complete when the summary is sent.	What share of follow-ups take place with discharge information present?	Artifact 04 § 10 Moves 4 and 7

SECTION 14

APPENDIX — EVIDENCE MAP

This map traces each major diagnostic claim to its evidence tier and representative sources, so that no structural claim rests on a single source class.

Evidence tier key

TIER	CLASS	DESCRIPTION
[1]	First-party NHS and government	NHS England and DHSC materials — stated promise, official workflow, contracts, programme pages, policy and legislative announcements
[2]	Independent structural oversight	HSSIB investigations; NAO and Public Accounts Committee reporting
[3]	Consequence evidence	NHS Resolution data; coroners' PFD reports and statistics; press reporting of PFD patterns. Pattern evidence, not findings of liability
[4]	Workforce and collapse-node evidence	RCGP surveys; Health Foundation projections; BMA analysis
[5]	Patient and hidden-burden evidence	Healthwatch England — treated as pattern evidence, not individual anecdote
[6]	Academic and peer-reviewed	BJGP, BMJ Open, Annals of Family Medicine, JMIR Medical Informatics
[7]	Structural inference	The author's frameworks applied to the evidence — the Four-Function Law; the governing diagnosis. Not independently evidenced

Claim-to-source map

DIAGNOSTIC CLAIM	EVIDENCE TIER	REPRESENTATIVE SOURCES
There has been no structural requirement for continuity retrieval at the point of care	[1][2]	HSSIB (2023): no specific contract requirement; no GP IT requirement to surface repeat presentations. DHSC response: 2024/25 contract requires continuity to be considered. 2025/26 contract: continuity incentive. No retrieval standard identified.
Corrections have addressed documentation and access, not retrieval at the decision	[1][2][7]	Public Accounts Committee (2013) on NPfIT; NHS England programme pages (SCR, shared care records, ConCR, NHS App); Single Patient Record announcements.
The patient is the de facto memory carrier	[2][5]	Healthwatch England (2025, n=1,800); HSSIB (2025) on over-reliance on patients and families.
Sensing, interpretation and authority are fused in the GP consultation	[2][4][6][7]	HSSIB (2023) on continuity systems and time to process information; RCGP GP Voice (2024); BMJ Open (2024) task analysis; Four-Function Law.
Discharge handover is a structural failure, not only a communication delay	[2]	HSSIB (2025): summaries created but not sent; one incident running around a year; system design and configuration problems.
The consequence layer is patterned, not random	[3][6]	Coroners Statistics 2024; The Guardian (Nov 2024) on 36 warnings; Isaac Onyeka and Regan Smith PFD reports; NHS Resolution CNSGP data; Annals of Family Medicine (2017); BJGP (2025).
The governing diagnosis	[1]–[7]	Convergent across all tiers. No single tier is sufficient alone. The structural inference is supported where the evidenced tiers are consistent with it, as they are in the evidence reviewed.

SECTION 14.1

SOURCE REGISTER

Representative sources by evidence tier, reviewed through October 2026. For live engagements, a full source log with access dates is available on request.

TIER	SOURCE	SPECIFIC ITEM	USED FOR
[1]	DHSC / HSSIB recommendations log	Government response to HSSIB R/2023/010: the 2024/25 GP contract requires continuity to be considered when responding to patients	Current contractual position
[1]	NHS England / The King's Fund	2025/26 GP contract: Capacity and Access Improvement Payment element to identify patients who would benefit most from continuity	Current contractual position
[1]	NHS England	Connecting Care Records programme page: successor to the national Shared Care Record programme; expected to finish March 2026	Shared care record status
[1]	DHSC / King's Speech (May 2026)	NHS Modernisation Bill announced to enable a single patient record viewable in the NHS App; also allows NHS England to be abolished	Current reform landscape

TIER	SOURCE	SPECIFIC ITEM	USED FOR
[1]	DHSC response to a coroner's report (June 2026)	Response on record-sharing between trusts, pointing to the Single Patient Record programme	Ongoing coroner concern; current reform
[2]	HSSIB (30 November 2023)	Continuity of care: delayed diagnosis in GP practices	Absence of contract and GP IT requirements
[2]	HSSIB (10 July 2025)	Electronic communications on patient discharge from acute hospitals	Discharge handover failure; reliance on families
[2]	Public Accounts Committee (September 2013)	The dismantled National Programme for IT: forecast cost £9.8bn; benefits "extremely disappointing"	Prior reform outcome
[3]	NHS Resolution	Annual Report and Accounts 2023/24: £2.8bn paid across all schemes; CNSGP 2,382 claims (+9%)	Consequence environment
[3]	NHS Resolution (August 2022)	CNSGP year-one report: 401 cases (2019/20); delay/failure to diagnose 43.5%	Consequence environment
[3]	Ministry of Justice (May 2025)	Coroners Statistics 2024: 713 PFD reports, up 25%	Consequence environment
[3]	The Guardian (11 November 2024)	36 coroners' warnings in 2024 on NHS information sharing	Consequence pattern
[3]	Coroners' PFD reports	Isaac Onyeka (East London, 2024-0132); Regan Smith (Suffolk, 2024-0479)	Documented cases
[4]	RCGP	GP Voice survey (July 2024): 76% patient safety compromised by workload; poll (September 2024): 42% unlikely to be in general practice in five years	Workforce strain
[4]	Health Foundation (2022)	GP workforce projections to 2030/31: ~8,800 FTE shortfall under current policy	Workforce outlook
[5]	Healthwatch England (1 May 2025)	The extent and impact of inaccurate NHS patient records (n=1,800)	Patient burden
[6]	Annals of Family Medicine (2017)	Continuity of primary care and emergency hospital admissions among older patients in England	Continuity and admissions
[6]	BJGP (2025)	Personal GP continuity improves healthcare outcomes: a systematic review	Continuity and outcomes
[6]	BMJ Open (2024)	Great expectations? GPs' estimations of time required to deliver BMJ's '10 minute consultations' — 2 min 38 s per consultation for reading notes, calling the patient and documenting	Consultation compression
[6]	JMIR Medical Informatics (2023)	A call to reconsider a nationwide electronic health record system — 11 million cross-system attendances (2017/18)	Record fragmentation
[7]	Four-Function Law (Forrester 2026)	Structural inference framework applied in Section 7	Functional diagnosis; not independently evidenced

SECTION 15

APPENDIX — DIAGNOSTIC RESOLUTION RECORD

DIAGNOSTIC RESOLUTION RECORD — EXEMPLAR

This page shows how the governing diagnosis can be formally resolved before implementation-level redesign begins. It is not represented as the mandatory contract form for every engagement, and no universal signature deadline is asserted here. Current practice is written-first and fixed-scope: the Diagnostic Artifact is delivered, the diagnostic frame is resolved and locked, and redesign begins only under a Full Structural Engagement. This public exemplar was not commissioned by the NHS, NHS England, DHSC or any associated body.

In a live engagement, the purpose of resolution is to prevent redesign being interpreted through an unaccepted or unstable diagnostic frame. The mechanism used to record that resolution may vary by engagement.

Resolution checklist

- ☐ **Governing diagnosis resolved**
  Leadership records that operational memory is not reliably present at the point of clinical consequence, that the GP consultation is the collapse node, and that the failure is architectural rather than a GP capacity or digital investment problem alone.
- ☐ **Diagnostic refusal conditions acknowledged**
  The Section 12 refusal conditions are accepted as constraints on redesign, not as optional optimisation items.
- ☐ **Dependency order accepted**
  The five-condition diagnostic order in Section 12 is accepted. Artifact 04 expands it into one implementation sequence.
- ☐ **Baselines agreed**
  Patient-reported history repetition; GP time on reconstruction; discharge information present before follow-up; emergency admissions for repeat presenters.
- ☐ **Full Redesign Artifact commissioned where applicable**
  Artifact 04 is commissioned only under a Full Structural Engagement after the diagnostic frame is locked.
- ☐ **Current-state verification**
  A live engagement verifies internal arrangements — for example local retrieval or alerting already in use — before treating a condition as absent.
- ☐ **Build boundary accepted**
  Stakeholders do not claim redesign success from partial implementation. Proof requires usable continuity to change behaviour under the original pressure.
- ☐ **Recapture risks noted**
  Leadership notes that redesign must not create a new form of the old failure: a record assembled for audit rather than retrieval, or a patient-facing feature that moves the burden back onto the patient.

Resolution signatures (exemplar)

Roles are exemplar allocations. The architectural requirement is an authority above local pressure; in practice, the relevant current bodies or their successors.

ROLE	NAME	SIGNATURE	DATE
Chief Executive, NHS England (or successor)			
Chief Digital and Information Officer			
National Director of Primary Care			
Chief Clinical Information Officer			

Where a live engagement uses a signed resolution record, the signed page records that the governing diagnostic frame has been resolved before implementation-level redesign begins.

CLOSING

CLOSING STATEMENT

> ***This artifact names the condition that public evidence makes visible: an architecture built to record events is being asked to deliver continuity it was not designed to provide. The NHS has built the audit layer. It has not yet built the operational layer. Until operational memory is placed at the point of clinical consequence — by design, not by patient recall — the same failure remains liable to reappear under new programmes, new contracts and new digital investment. The redesign is real only when the same pressure produces a different response: when the patient no longer has to repeat their history.***

WHAT THIS DEMONSTRATES

This public diagnostic exemplar demonstrates the diagnostic logic, evidence tiering, governing diagnosis and correction dependency order, derived entirely from public evidence. It is not sufficient for implementation. The restricted Full Redesign Artifact (Artifact 04) contains one replacement architecture, its transition sequence, refusal invariants, recapture gates, admissibility conditions and builder handoff. This document demonstrates the method. The Full Redesign Artifact specifies an implementation.

This artifact was produced by Jamie Forrester, Independent Systems Architect, Edinburgh, UK. It is based entirely on publicly available information and was produced independently — it was not commissioned by the NHS, NHS England, DHSC or any associated body. The diagnostic logic, structural interpretation and governing diagnosis are the author's own.

For the Full Redesign Artifact (Artifact 04) or the Translation Artifact (Artifact 01), contact:
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